

Eastbrook General Hospital — Department of Surgery

OPERATIVE REPORT

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Christine V. Donnelly	DOB: 10/17/1981	MRN: 8821047
Date: October 25, 2024	Provider: Dr. Anthony Walsh, MD, FACS — General Surgery	

OPERATIVE SUMMARY

Pre-op Diagnosis:	Symptomatic cholelithiasis with recurrent biliary colic	Post-op Diagnosis:	Symptomatic cholelithiasis with recurrent biliary colic (confirmed)
Procedure:	Laparoscopic cholecystectomy with intraoperative cholangiogram (IOC)	Surgeon:	Anthony Walsh, MD, FACS
Assistant:	Sarah Kim, MD (PGY-3 Resident)	Anesthesia:	General endotracheal (Dr. R. Nguyen, CRNA)
Estimated Blood Loss:	< 10 mL	Complications:	None
Specimens:	Gallbladder to surgical pathology	Disposition:	PACU, then same-day discharge (home)

OPERATIVE TECHNIQUE

The patient was taken to the operating room, placed supine, and underwent general endotracheal anesthesia without difficulty. The abdomen was prepped and draped in the standard sterile fashion. A Veress needle was placed in the left upper quadrant using the Palmer's point approach, and a pneumoperitoneum of 15 mmHg was established without difficulty.

A 12mm umbilical port was placed under direct visualization with a 0-degree laparoscope. An additional 5mm epigastric port was placed under direct vision. Under laparoscopic guidance, two 5mm ports were placed in the right upper quadrant — one subcostal and one lateral.

The liver was elevated and the gallbladder fundus was grasped with a laparoscopic grasper. Dissection of the hepatocystic triangle was performed with electrocautery and blunt dissection. The peritoneum overlying the triangle of Calot was cleared anteriorly and posteriorly. Critical view of safety (CVS) was achieved: the hepatocystic triangle was cleared of fat and fibrous tissue, the lower third of the gallbladder was separated from the liver bed, and two and only two structures — the cystic artery and cystic duct — were seen entering the gallbladder.

Intraoperative cholangiogram was performed by placing a 4Fr cholangiocatheter through a cystic ductotomy. Fluoroscopic images demonstrated free flow of contrast into the duodenum with normal opacification of the biliary tree. No filling defects to suggest choledocholithiasis. No ductal injury.

The cystic duct was doubly clipped proximally and divided. The cystic artery was doubly clipped and divided. The gallbladder was dissected off the liver bed in the plane of the submucosal layer using electrocautery. The gallbladder was placed into an EndoBag specimen retrieval device and extracted through the umbilical port site without spillage. The liver bed was inspected — dry. No bile or blood leak. Fascial closure performed at the 12mm umbilical site with 0-Vicryl. Skin closed with 4-0 Monocryl subcuticular sutures. Dermabond applied to all ports. Patient tolerated the procedure well.

POSTOPERATIVE ORDERS

Diet:	Clear liquids on arrival to PACU, advance to regular diet as tolerated	Activity:	No lifting >10 lbs x 2 weeks. Return to office work in 2-3 days
Pain:	Acetaminophen 500mg q6h + ibuprofen 400mg q8h PRN. Oxycodone 5mg PRN breakthrough	Wound:	Steri-strips to all port sites. Shower in 24 hours. No submerging until cleared
Follow-up:	Surgical clinic in 2 weeks (Dr. Walsh). Call sooner for fever, jaundice, or increasing pain		

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